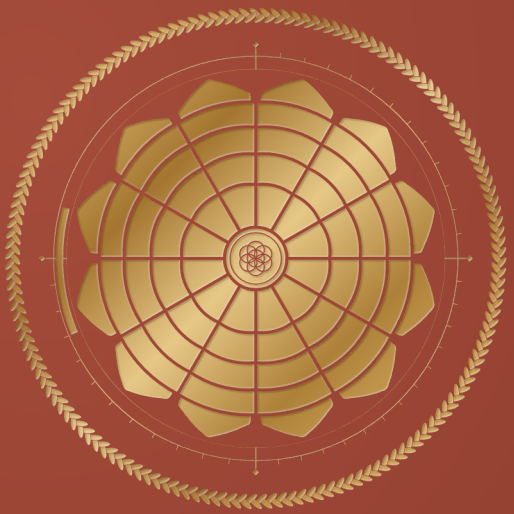
CARDS FOR PREGNANCY AND BIRTH



The Pregnancy and Birth Mandala

Companion book to the card deck

THROUGH THE LENS OF PRE- AND PERINATAL EDUCATION

ERIN SINGLETON

CARDS FOR PREGNANCY AND BIRTH

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Key to the mandala

THE RINGS, FROM THE CENTER OUT

- **The center.** Your baby, the new life beginning, growing, and expanding.
- **The baby.** Her growth, and what she may be sensing.
- **The mother.** Your body, your emotional life, and caring for yourself.
- **The circle of support.** How the people around you can hold you both, and be held too.
- **The educator.** Possible imprints, the baby's question, and what helps.
- **The practices.** The outer petals: for connecting with your baby, and for your own healing.

AROUND THE CIRCLE, CLOCKWISE FROM THE TOP



First trimester
Months 1 to 3



Second trimester
Months 4 to 6



Third trimester
Months 7 to 9



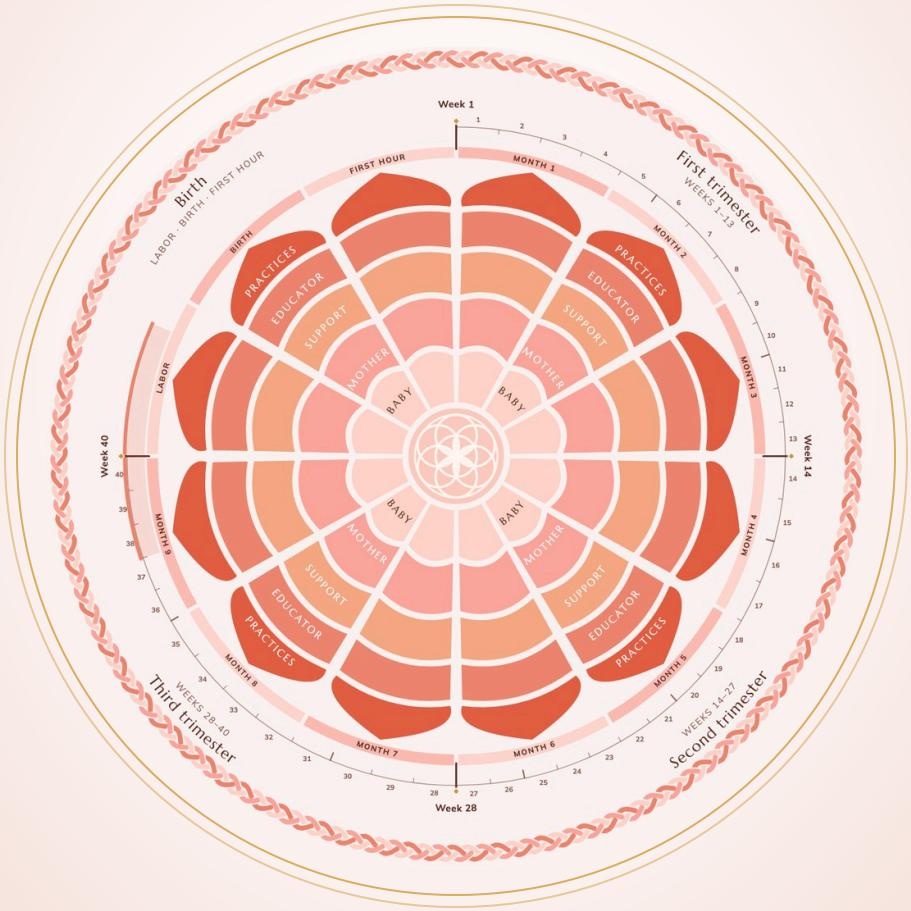
Birth
Labor, birth, first hour

AROUND THE EDGE

- **The time wheel.** Each month a span of color, and the forty weeks numbered, from week 1 at the top.
- **The braid.** Three strands, for the baby, the mother, and the support around her, holding the circle together.

See it come alive. The interactive mandala opens every month in detail, and with a due date entered it shows today, the birth window, and the moons of your pregnancy.
erinsarita.github.io/prenatal-mandala

THE PREGNANCY AND BIRTH MANDALA



WELCOME

A map for the journey, with a child at its center

Pregnancy is often described in weeks, tests, and appointments. This deck invites you to see it another way: as a journey you and your baby take together, from the first days to the first hour after birth.

In pre- and perinatal education, the baby is understood as aware from the very beginning. She is taking in her world, and what she experiences in the womb, at birth, and in her first hours may leave imprints that shape her long after. This is not a weight to carry. It is an invitation to slow down, to notice, and to connect.

The cards come from a mandala, a circle organized around a center. At its center is your baby, the new life growing within you. Around her is you, and around you is your circle of support, the people who hold you so you can hold her. Then comes what is good to know, and on the outer petals are practices for connecting with her and for your own healing.

The pages that follow lay the foundation: who your baby already is, how your world becomes hers, what an imprint is, your nervous system and your feelings, and how to speak with her. Then come the months, one card at a time, and people who can walk with you further.

> There are no secrets you can keep from your baby, so talk to her, and more importantly, listen.
>
> Karen Strange, Simple Tools for Mothers

START HERE

A conscious pregnancy

Pregnancy is often lived as a series of tests and dates, with the baby treated as a passenger until birth. Pre- and perinatal education offers another way: your baby is already here, sensing, learning, and taking in her world, and the months before birth are the beginning of your relationship.

A conscious pregnancy means growing aware of her, and aware of yourself: your feelings, your body, the world around you, and how all of it reaches her.

WHAT THE DECK IS FOR

The cards turn this into something you can do. Each holds a picture of her month and of yours, how the people around you can hold you both, the imprints that may begin to form and the question that may begin to emerge for her, and practices for connecting with her and for your own healing.

HOW TO USE THIS BOOK

> **Begin with the foundations.** Read them at your own pace. They explain why the practices matter.

> **Then go to your month.** Read its page here, and keep its card close.

> **Return whenever you need to.** The foundations are here for hard days, and the "Going deeper" pages at the back are here when you want more.

> **If your path is harder,** through loss, conceiving with help, a hard birth, or grief, begin with page 59.

Your baby is already here

Long before she is born, your baby is moving, sensing, sleeping and waking, and learning. She is not waiting to begin. Her life, and her relationship with you, have already started.

Pre- and perinatal psychology takes the view that she is a feeling, sensing person from the very beginning: aware in her own way, and taking in how she is received. David Chamberlain called the prenate "a conscious, sentient being."

Science can measure her responses: a quickening heart, a turn toward a voice, a startle at a sound. It cannot yet measure her inner experience, and how and when awareness begins is still debated.

A question to carry. What would change if you met her, from today, as someone and not something? Not as a project to manage, but as a person getting to know you.

What she senses, and when

Times are approximate, and every baby develops in her own way.

From 7 to 8 weeks	**Movement and touch.** She begins to move, long before you can feel it. The first touch receptors form around her mouth, and by about 20 weeks most of her body can sense touch.
From 10 to 15 weeks	**Taste.** Taste buds form, and she swallows amniotic fluid flavored by what you eat.
From 18 to 25 weeks	**Hearing.** The first sounds reach her, your voice clearest of all, and by about 25 weeks she responds to sound.
From 26 to 28 weeks	**Light.** Her eyes open, and she can sense bright light through your belly.
From 28 to 32 weeks	**Dreaming sleep.** Active, dreaming sleep appears, and she spends much of each day in it.
The last weeks	**Learning and memory.** She grows used to sounds she hears often, knows your voice from a stranger's, and after birth prefers the voices, songs, and stories she heard in the womb.
All along	**Your states.** Through your heartbeat, breath, movement, and the chemistry that crosses the placenta, she lives inside the rhythm of your days.

You are her first world

Before she meets the world, she lives inside yours. Everything that reaches her comes through you: what you feel, what you think, what you take in, the rhythms of your days, and the people around you.

YOUR INNER WORLD

Emotional

Your feelings change your body's chemistry. Stress hormones such as cortisol, and calming ones such as oxytocin, rise and fall with your states, and some reach her through the placenta. Your heartbeat and breath, her constant background, quicken and settle with you.

Mental

Your thoughts, beliefs, and expectations shape your feelings and your choices: whether you see her as someone, what you expect of birth, the stories you tell yourself. A fearful story and a trusting one feel different in the body, and so to her.

You do not need to feel calm all the time. Everyday stress is part of life, and the placenta buffers much of it. What matters most is the overall climate, and how often you find your way back.

The world around you

Physical

Nourishment, water, rest, movement, and fresh air build her body, and what you take in, including alcohol, nicotine, and some medications, reaches her too. The sounds, light, and pace of your surroundings become part of her world.

Relational

Your partner, family, friends, and care providers, and whether you feel safe, supported, and respected among them. Conflict and kindness both reach her through you.

The wider field

Work, money, culture, and the times you are pregnant in. Some of this is beyond your control. Noticing it, and asking for support, is within it.

HOW IT REACHES HER

> **Through the placenta:** nutrients, hormones, and some of what you take in.

> **Through your rhythms:** heartbeat, breath, movement, and sleep.

> **Through sound:** your voice, and the voices around you.

> **Over time, through her genes:** early surroundings can change how genes are switched on and off, a field called epigenetics, one way experience is carried forward (page 13).

Experience and the genes

Every cell in your baby's body carries the same genes. What differs is which genes are speaking and which stay quiet. **Epigenetics** is the study of the small chemical marks that set this, switching genes on and off without changing the genes themselves. Think of a piano: the keys stay the same, but the music changes with how they are played.

Those marks respond to the world. Nourishment, stress, calm, and care all help write them, and some are written very early, while she is still in the womb.

WHAT THE RESEARCH SHOWS

> **Care shapes stress.** Rat mothers who licked and groomed their pups more raised calmer offspring, through marks on a gene for a stress-hormone receptor. Pups raised by a different mother took after the one who raised them (Weaver and colleagues, 2004).

> **Before birth.** Babies conceived during the Dutch Hunger Winter of 1944 and 1945 carried different marks on a growth gene six decades later (Heijmans and colleagues, 2008).

> **A mother's mood.** More depressed mood in pregnancy was linked with more of these marks on the same stress-hormone receptor gene in newborns (Oberlander and colleagues, 2008).

A careful note. Much of this research is young, and many studies are small. In people it shows links, not causes, and every baby is her own.

Not destiny

ACROSS GENERATIONS

When a mother carries a daughter, the eggs that may one day become her grandchildren are already forming inside that baby. For a time, three generations share one body.

Rachel Yehuda found differences in marks on a stress-related gene in Holocaust survivors, and different ones in their grown children (Yehuda and colleagues, 2016). These findings are still debated, yet they echo what many families know: patterns can pass from one generation to the next.

MARKS CAN CHANGE

Epigenetic marks are not fixed. In the animal studies, they could be reversed, and care, connection, and calm keep shaping them throughout life. What was passed on can be met, and something new can be passed on instead.

> **Nourish and rest.** Small, steady steps count.

> **Return to calm.** Everyday stress is buffered. It is long, unrelieved stress, without support, that matters most.

> **Let yourself be held.** Your circle of support is part of her world.

> **Repair.** Each return, each "I am here," is part of what she learns.

> **Tend your own story.** Your healing is part of what you pass on.

A reason for care, not worry. This is not a list of things that could go wrong. It is a reminder that what you do each day reaches her, and that it is not too late to begin.

The hormones of pregnancy

Hormones are your body's messengers. In pregnancy they shift more, and faster, than at almost any other time in life, building her body, preparing yours, and coloring how you feel.

hCG	Made from the first days, it is what a pregnancy test finds, and it is behind much of early nausea.
Progesterone	Keeps the pregnancy steady and relaxes smooth muscle. It can bring deep tiredness, and a slower, inward calm.
Estrogen	Grows the womb and its blood supply, and can heighten smell and feeling.
Relaxin	Softens ligaments and joints, making room for her and for birth.
Prolactin	Rises steadily to prepare your breasts for milk, and is linked with nurturing.
Cortisol	Rises naturally through pregnancy and helps her lungs and organs mature. The placenta buffers much of yours; it is long, unrelieved stress that matters most.
Oxytocin	The hormone of calm and closeness. Late in pregnancy your womb grows many more receptors for it, ready for labor.

With so much shifting, feelings can run bigger and change faster. This is your body at work, not a flaw in you.

The hormones of birth and bonding

Sarah Buckley describes four hormone systems that carry mother and baby through birth (Buckley, 2015).

Oxytocin	Brings the rhythm of contractions, and love. It peaks at birth and in the first hour, skin to skin.
Endorphins	Your own pain relief, carrying you inward as labor deepens.
Adrenaline	Early in labor, fear or disturbance can slow things down. At the very end, a surge gives strength for the final pushes, and makes your baby alert to meet you.
Prolactin	Begins milk and mothering, and brings a quiet calm.

These flow best when you feel safe, warm, private, and unobserved. Your baby has birth hormones of her own too, which protect her through labor and prepare her lungs.

THE CHEMISTRY OF CONNECTION

Oxytocin is released by touch, warmth, eye contact, a loving voice, skin to skin, and breastfeeding, in both of you. Each time, the bond is laid down a little deeper. Partners change too: in fathers who care closely for their babies, oxytocin rises and testosterone falls (Gordon and colleagues, 2010; Gettler and colleagues, 2011).

The "oxytocin moments" on the cards are practice for this: pleasure and closeness on purpose, so the pathway is well worn before she arrives.

What is pre- and perinatal education?

Prenatal means before birth, and *perinatal* means around birth. Pre- and perinatal education draws on pre- and perinatal psychology, the study of how experiences from conception through pregnancy, birth, and the first year shape a person's body, relationships, and sense of self.

Its central idea is simple: your baby is a feeling, sensing participant in her own beginning, not a passenger. She responds to touch, sound, and your inner states long before birth, and she is learning about the world she is about to join.

WHERE IT COMES FROM

1924	Otto Rank's *The Trauma of Birth* proposes that being born leaves a lasting mark.
1930s	Konrad Lorenz describes *imprinting* in young geese, the word this field later borrows.
1960s–70s	Frank Lake and Stanislav Grof notice memories of womb and birth surfacing in adult therapy.
1981–83	Thomas Verny publishes *The Secret Life of the Unborn Child* and founds the association that becomes APPPAH.
1988	David Chamberlain's *Babies Remember Birth* gathers birth memories from children and adults.
Since	Research on fetal programming, attachment, and the developing nervous system shows how early environments shape lifelong health.

Today APPPAH, the Association for Prenatal and Perinatal Psychology and Health, trains educators and practitioners to bring this understanding to families. This guide grew out of that training.

Why it matters, for the benefit of all

Seeing pregnancy through this lens turns it from something that happens to you into a relationship you take part in. Knowing what may leave an impression lets you offer more of what nourishes, and meet hard moments with repair instead of guilt.

> **For your baby.** A welcome felt from the start, a nervous system that learns calm from yours, and early experiences of being heard.

> **For you.** Less fear and more confidence, a way to connect before you can hold her, and room for your own story to heal.

> **For your partner and family.** A real part from the beginning: a voice she comes to know, a hand that answers her movements.

> **For those who care for you.** A shared language for guarding the birth space and keeping mother and baby together.

> **For the generations to come.** Patterns pass from parent to child. Each gentle beginning, and each repair, changes what is passed on.

Putting it into practice. Each card turns this understanding into small, daily acts: talking to her, answering her movements, settling yourself, guarding the birth space, and repairing after hard moments.

This is not about getting everything right. It is about noticing, connecting, and repairing, which every family can do.

What is an imprint?

The word comes from Konrad Lorenz, who watched newly hatched goslings follow the first moving figure they saw, often Lorenz himself, and keep following. A first experience, met in a sensitive window, set a lasting pattern.

In pre- and perinatal education, an imprint is an impression left by early experience: conception, life in the womb, birth, and the first hours and days. Because it comes before words, it is held as body memory, in the nervous system and in patterns of feeling and response, rather than as a story she can tell.

Imprints shape her first answers to quiet questions: *Am I welcome? Is the world safe? When I reach out, will someone meet me?* Each month in this guide, and the back of each card, names a question that may begin to emerge for her then. It is not settled in that month: it stays open, and what she lives through afterward, including every repair, keeps answering it.

Three things to hold

Imprints can nourish. Welcome, calm, touch, and being spoken to leave impressions too. Most of this guide is about offering more of these.

An imprint is not a sentence. Later relationships, repair, and healing can reshape early patterns, at any age.

You carry imprints too. Pregnancy can stir your own earliest story. That is why every card holds a practice for your own healing.

Memory before words

Memory comes in two kinds. **Explicit memory** is what we can recall and put into words: facts, events, the story of a day. **Implicit memory** is held without any sense of remembering: in the body, in emotions, in what we expect and how we react.

Explicit memory depends on the hippocampus, which keeps maturing through the first years of life. This is why most of us recall little before age three or four. Implicit memory is at work much earlier, before birth.

WHAT RESEARCH SHOWS

> Newborns prefer their mother's voice to other voices, a voice they came to know through the womb wall (DeCasper and Fifer, 1980).

> Babies whose mothers read one story aloud in the last weeks of pregnancy preferred that story after birth (DeCasper and Spence, 1986).

> Flavors from a mother's meals reach the amniotic fluid. Babies who tasted carrot this way later took to carrot-flavored cereal more readily (Mennella and colleagues, 2001).

She is learning before she can remember. Her early experiences are not lost: they are carried as implicit patterns of feeling and response (Siegel).

Why this guide says "may." The imprints on these pages are possible, not certain. Every baby meets her experiences with her own temperament, and the people around her shape what those experiences come to mean.

Trauma and resilience

In this lens, trauma is defined less by the event than by what happens inside: an experience that is too much, too fast, or too soon, with too little support to meet it (Levine). The nervous system stays braced, as though the danger has not passed.

Not every hard experience becomes trauma. Researchers describe a range of stress: **positive** stress, brief and manageable; **tolerable** stress, buffered by caring relationships; and **toxic** stress, long, intense, and unbuffered (Center on the Developing Child, Harvard).

RESILIENCE

Resilience is the capacity to meet difficulty and find the way back to balance. It is not a fixed trait. It grows through relationship: being soothed, being understood, and knowing repair after rupture.

For your baby, resilience begins with co-regulation: her young nervous system borrows steadiness from yours. For you, it grows through support, rest, and tools you can reach for in the moment. Each return to calm is practice, for both of you, in the way back.

The pages that follow give you a map for this: your nervous system, and your window of tolerance.

> Each "Imprints and what helps" section holds both sides: what may be hard, and what builds resilience.

Your nervous system: an introduction to polyvagal theory

Polyvagal theory, introduced by Stephen Porges in 1994, describes how the autonomic nervous system responds to safety and danger. It offers a simple map of three states.

Safe and connected

Breath is easy, face and voice soften, and you can rest, play, and bond. Oxytocin flows most freely here, in pregnancy and in labor.

Mobilized

Heart and breath quicken, ready to fight or flee. Helpful in bursts, tiring when it lingers.

Shut down

When danger feels overwhelming, the body may conserve: numbness, collapse, feeling far away.

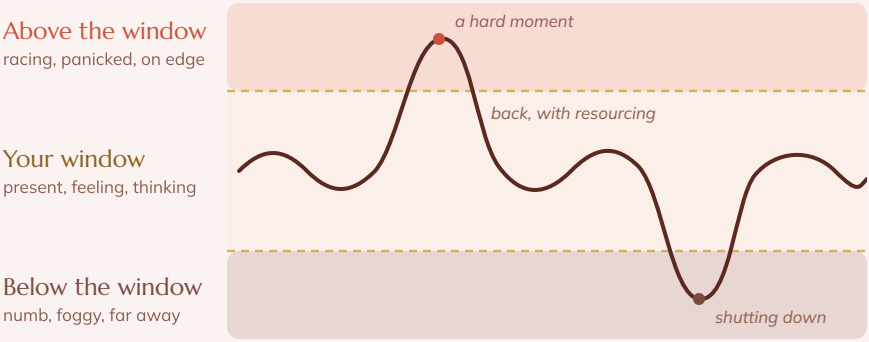
Porges calls the body's constant, wordless scanning for safety *neuroception*. Your baby senses your state through your heartbeat, breath, voice, and chemistry, and her system begins to tune to yours.

Trauma can leave a nervous system stuck in mobilized or shut-down states. Resilience is the flexibility to move through them and find the way back to safety.

Polyvagal theory is widely used by therapists and educators, and some of its physiological details are still debated by researchers. Its map of states is offered here as a practical guide.

Your window of tolerance

Daniel Siegel describes a *window of tolerance*, which some call a window of capacity: the range in which you can feel strong feelings and still think, connect, and respond. Everyone has one, and it widens and narrows with sleep, support, hormones, and stress.



Above the window

A racing heart, a tight chest, panic, irritability, a mind that will not settle. This is the mobilized state.

Inside the window

Present, able to feel and to think. You can notice your baby, and respond rather than react.

Below the window

Numb, foggy, flat, far away, wanting to shut down. This is the shut-down state.

Pregnancy can narrow the window: tiredness, nausea, shifting hormones, and old memories stirring. This is not a failing. It is a signal to resource.

Widening your window

> **Notice where you are.** A few times a day, ask: am I above, inside, or below my window right now? Naming it already helps.

> **If you are above it,** slow the out-breath, feel your feet, let your eyes rest on something pleasant, lower the noise and light, and move gently.

> **If you are below it,** move a little, splash cool water on your face, sit upright, hold something warm, reach for a familiar voice, and name five things you can see.

> **Over time,** sleep, nourishment, people who feel safe, the practices on the cards, and therapy if old pain keeps the window narrow.

HER WINDOW GROWS INSIDE YOURS

A baby cannot yet calm herself. She borrows steadiness from you, first through your body, later through your arms and voice. This is called co-regulation. Each time you find your way back into your window, you are showing her the way.

In labor, a wide window lets you stay present through great intensity. The practices you build now are preparation for then.

Feelings: emotional intelligence in pregnancy

Emotional intelligence is the ability to notice, name, and work with feelings, your own and other people's. In pregnancy it becomes a gift to two people at once.

FEELINGS ARE INFORMATION

Joy, fear, grief, anger, and ambivalence each have something to tell you. They are not verdicts on you as a mother.

NAME IT TO TAME IT

Daniel Siegel's phrase for a simple finding: putting a feeling into words can calm the brain's alarm (Lieberman and colleagues, 2007). "I feel scared" is already a step toward steadiness.

MIXED FEELINGS ARE NORMAL

Many parents feel two things at once: wanting this baby and grieving the life before, love and fear, excitement and dread. Both can be true.

Your feelings, and hers

Your baby feels what you feel, through your body, but she cannot yet tell whose feeling it is. Pre- and perinatal practice offers a simple way to help: tell her.

> "I am angry right now, and it is not about you."

> "I am sad today. You did nothing wrong."

> "That was a hard moment. I am back now, and I am glad you are here."

This begins what Karen Strange calls healthy separateness: she can be with your feeling without carrying it as her own.

When feelings are big. If sadness, worry, or numbness last two weeks or more, or you have thoughts of harming yourself, reach out. Depression and anxiety in pregnancy are common and treatable, and page 66 lists where to turn.

Your own healing

Carrying a child often brings your own beginning closer. As you prepare to meet her, parts of your own story may rise to meet you.

WHAT MAY COME UP

> Your own birth, and what you know or sense about it.

> Your mother, your childhood, and the ways you were cared for, or were not.

> Earlier pregnancies, losses, or births, including one you may be looking back on now.

> Old fears, griefs, or patterns in your relationships, sometimes felt first in the body.

None of this means something is wrong. It is a natural part of becoming a parent, and often a sign that something is ready to be met.

HOW THE HEALING PRACTICES HELP

The back of every card holds a practice for your own healing: reflecting on your own beginning, telling your story, meeting your younger self with kindness, and offering yourself the repair you are learning to offer her. What is felt and soothed in you is less likely to be passed on.

A time for transformation

Pregnancy is a time of deep change. Your hormones, body, sleep, relationships, and sense of who you are all shift at once. Anthropologist Dana Raphael named this passage *matrescence*: the becoming of a mother, as profound as adolescence.

The brain changes too. Imaging studies show that pregnancy reshapes areas involved in understanding others, changes that last for years (Hoekzema and colleagues, 2017). Many in this field see this openness as a rare window, a time when old patterns are more able to shift.

So what rises now can be met as an opportunity. Healing in pregnancy is not only for you. It changes the world your baby grows in, and what is passed on.

IF YOU NEED MORE SUPPORT

> **People who can walk with you,** page 65: practitioners and approaches that work with early experience.

> **My circles of support,** page 67: a place to keep their names and numbers.

> **Memories that return,** page 63: how early memory can surface, and how it may be held.

> **Help, reading, and sources,** page 66: if you need support now.

Resourcing yourself in the moment

A resource is anything that helps your nervous system feel a little steadier: a person, a place, a memory, a sensation in your body, something you can hold. Resourcing is turning toward these on purpose.

Knowing your nervous system widens your awareness. When you can notice "I am speeding up" or "I am drifting away," you have a choice, and the noticing itself is a step back toward presence, through pregnancy, labor, and the first hours.

TOOLS FOR THE MOMENT

> **Orient.** Let your eyes move slowly around the room and rest on something pleasant.

> **Ground.** Feel your feet on the floor, or your weight held by the chair.

> **Lengthen the out-breath.** Breathe in, then let the breath out a little longer.

> **Touch.** A hand on your heart or your belly, for you and for her.

> **Hum or sing.** Gentle sound on a long exhale can help your system settle.

> **Reach out.** A familiar voice or a hand to hold is among the strongest resources there are.

The back of each card holds practices with time set aside to connect: with your baby, with your partner, and with yourself. Return to them whenever you need resourcing.

A daily check-in

A few minutes, once or twice a day, ties everything in this book together. It is the bridge to the cards.

1. **Pause.** Stop where you are, and feel your feet or your seat.
2. **Breathe.** A few slow breaths, with the out-breath a little longer.
3. **Notice yourself.** Where am I in my window? What am I feeling? Name it.
4. **Turn toward her.** A hand on your belly. Notice her: movement, stillness, the weight of her.
5. **Speak.** Good morning, what the day holds, what you feel and whose it is.
6. **Listen.** Pause, and notice what comes: a movement, a sense, a feeling.

Then turn to your card, and choose one practice for the day.

Why speak to a baby who has no words?

Many practices on the cards ask you to talk to your baby: to tell her what is happening, what you feel, and what you hope. It can feel odd at first. She cannot understand words yet. So what reaches her?

WHAT REACHES HER

> **Your voice.** From mid-pregnancy she hears, and your voice, carried through your own body, is the loudest and clearest in her world.

> **Rhythm and warmth.** Long before meaning, she takes in the music of speech: its pace, its tone, its rise and fall.

> **Your body.** When you speak tenderly, your breath slows, your heart steadies, and your muscles soften. She lives inside these changes.

> **Your chemistry.** Warmth and connection bring oxytocin, the hormone of calm and closeness, and ease the stress hormones that matter most when they linger.

In pre- and perinatal education, words are understood as carriers of your attention, your intention, and your feeling. She may not know what "you are safe" means. She can feel what it is like when you mean it.

For partners. Your voice reaches her too, from just outside the womb. Babies come to know the voices they hear often, so a partner who speaks to her before birth is already familiar when she arrives.

She is listening

WHAT RESEARCH SHOWS

> Before birth, babies' heart rates quicken to a recording of their mother's voice, and slow to a stranger's (Kisilevsky and colleagues, 2003).

> Newborns prefer their mother's voice, and a story read aloud to them in the womb (DeCasper and Fifer, 1980; DeCasper and Spence, 1986).

> Newborns' cries already carry the melody of the language spoken around them (Mampe and colleagues, 2009), and they recognize sounds heard often before birth (Partanen and colleagues, 2013).

> Mothers who sang lullabies through pregnancy and after birth reported a stronger bond, and their newborns cried less (Persico and colleagues, 2017).

There is an intelligence in her that we cannot fully grasp, and research cannot yet say how much of your meaning she receives. What we can see is what it brings: she listens, comes to know your voice, and responds, and speaking to her changes you as well.

That is where its importance lies. Each time you speak to her, the relationship grows. She is seen, she is heard, she is accounted for, and the connection you build becomes a resource you can both return to.

HOW TO BEGIN

Start small: good morning, her name, a hand on your belly. Tell her what you are about to do. Name what you feel: "I am worried today, and it is not about you." Invite your partner to speak to her too. Then pause, and notice. She may answer with a move.

Each card offers words you can say to her. Take them as a starting place, and make them your own.

Your part in this

This knowledge is not meant to add worry. It is meant to give you back your part. Much of pregnancy is beyond anyone's control. Your part is the relationship, and the choices that shape it.

> **Connection.** You can turn toward her every day, in small ways.

> **Awareness.** You can notice your own states, and find your way back to steadiness.

> **Choice.** You can ask questions, choose your care and your support, and give or withhold consent.

> **Voice.** You can speak to her, and speak up for her and for yourself.

> **Repair.** When hard moments come, you can return, name what happened, and reconnect.

It is not too late. Whatever has already happened in this pregnancy, or in your own beginning, connection and repair can begin today. This is not about perfection. It is about presence.

For partners

Your baby is getting to know you too. A partner's voice, touch, and steadiness reach her, and your support shapes the mother's world, which is her world.

> **Speak and sing to her.** A voice heard often before birth is familiar after it.

> **Answer her movements.** A hand on the belly when she kicks, and a word in reply.

> **Steady the mother.** Your calm helps widen her window. Take on what you can, listen without fixing, and protect her rest.

> **Be part of the decisions.** Learn the choices ahead, and help speak up for what she wants.

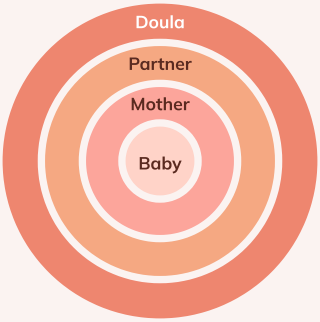
> **Meet your own story.** Becoming a parent can stir your own beginning too. The healing practices are for you as well.

Partners have their own feelings in pregnancy, including worry, distance, or a sense of being on the outside. Naming them, and finding support, is part of the work.

Layers of support

Ray Castellino, a pioneer of prenatal and birth therapy, taught that everyone who gives support needs support too. You hold your baby. The people around you hold you. And they need holding as well.

He found that the mother and the baby each do best with at least two layers of support around them. Your baby's first layer is you, and her second is whoever holds you.



An example of two layers of support, for both your baby and you.

WHAT SUPPORT CAN LOOK LIKE

Rest and meals, company at appointments, a listening ear that does not rush to fix, help at home, and someone who stays through the birth. It can come from a partner, family, friends, a doula, a midwife or doctor, or an educator.

Asking is part of it. Many people find it hard to ask for help. Each small ask makes the next one easier. Page 67 is a place to write down who is in yours.

Your circle, month by month

For each month, one way the people around you can hold you both, with your baby at the center.

MONTH 1
Let the people close to you ease your load and meet your news with care. Your rest is her first world.

MONTH 2
Ask for help through the nausea and fatigue, and let the people around you welcome her with you.

MONTH 3
Have someone with you at screenings and while you wait for results. A calmer home reaches her too.

MONTH 4
Invite someone to notice her movements with you, and to shield you from frightening birth stories.

MONTH 5
Let more loving voices reach her, and bring someone with you to the anatomy scan.

MONTH 6
Ask for real rest, and let others take on tasks. The calm rhythm of your home is the world she feels.

MONTH 7
Choose people who hear your fears without adding to them, and help your partner ready their voice for her.

MONTH 8
Plan help for after the birth, and keep your bond with your partner warm. She is held inside it.

MONTH 9
Let someone share the waiting and answer the questions, so you can turn inward and trust her timing.

LABOR
Have someone stay the whole way and guard a quiet space, telling her, too, what is happening.

BIRTH
Keep a familiar voice with you and with her, so neither of you is alone if plans change.

FIRST HOUR
Protect a quiet golden hour, so she is welcomed skin to skin, and by more than one person.

How each month is read

Every month in this guide follows the same pattern, moving from the center of the mandala outward.

Your baby

Her growth, and what she may be sensing and taking in. Read it as a description of someone, not something.

You

Your body, hormones, and feelings. Your inner states are her first environment, so caring for yourself is caring for her.

Your circle

How the people around you can support you, so you can support her. Each of you needs at least two layers of support.

Imprints and what helps

What the month's experiences may leave as an impression, what tends to help, and then the question that may begin to emerge for her. These are possibilities to notice, not predictions.

To explore further

The teachers and research behind the month, for when you want to read more deeply.

The back of each card carries the outer petals: practices for connecting with your baby, and one for your own healing.

Three questions to carry. What might she be experiencing? What am I experiencing? What would help us both?

Your card, front and back



FRONT



BACK

1. The month, and where it falls in the weeks of pregnancy.
2. The small wheel: where this card sits on the mandala.
3. **Your baby:** her growth, and what she may be sensing.
4. **You:** your body, hormones, and feelings.
5. **Your circle:** how the people around you can hold you both.
6. **Good to know:** her world, the possible imprints, and a note for you.
7. **Practices** for connecting with her, with steps to follow.
8. **For your own healing:** a practice for your own story.
9. **Baby's question:** a question that may begin to emerge for her, and why.
10. The teachers and research behind the card.

Seven threads behind every card

Most practices on the cards grow from Karen Strange's *Simple Tools for Mothers*. Knowing the threads behind them helps you make the practices your own.

1. **You are her regulator.** She forms around what you feel and experience. When you settle, she learns how settling feels.
2. **Slow down.** Grounding brings you into your body; pacing matches her slower rhythm. Her brain waves are six to ten times slower than yours.
3. **Talk to her, and listen.** Tell her what you will do before you do it, tell her what is going on, tell her what you would like, and tell her the story of what happened.
4. **Your feelings are yours.** She feels what you feel. Telling her "this is about me, not you" begins healthy separateness.
5. **Oxytocin moments.** Pleasure on purpose wires her system to move from stress back to calm, and gives you a groove to return to.
6. **Rupture and repair.** Hard moments come into every relationship. Returning, naming what happened, and reconnecting builds deeper trust.
7. **Someone for her, too.** A "baby doula" follows your baby's journey through birth, the way a doula supports you.

> It is not just what you do or say that matters, but how you are on the inside.
>
> Karen Strange

One card at a time

1. **Find your card.** Choose the card for the month you are in. The small wheel in its corner shows where it sits on the mandala.
2. **Read the front.** It holds a short picture of your baby, of you, of your circle of support, and of what is good to know this month.
3. **Turn it over.** The back holds practices for connecting with your baby, with steps to follow, and one practice for your own healing.
4. **Keep it close.** Set the card where you will see it: by your bed, on the fridge, in your bag. Return to its practices through the month.
5. **Look ahead.** Read the next card a little before its month begins. Read the three birth cards early in the third trimester, so you have time to practice.

A few gentle notes

Share them. Many practices are for partners, too. Read the cards together.

Go at your pace. Skip a practice that does not feel right. Return to it later, or not at all.

Write it down. A small journal beside the deck can hold what you notice and what surfaces.

Ask for help. If a practice stirs more than you can hold, page 65 lists people who can help.

Every practice is a way back

The practices on the cards help grow good imprints: welcome, safety, being met. They are also something more. If a hard moment has already happened, the same practices become a way back. It is not too late, and repair can begin today.

RUPTURE AND REPAIR

A rupture is a break in connection: a fright, a loud argument, a hard day, a birth that changed course. Ruptures come into every relationship. Research with mothers and babies finds that most everyday moments between them are small mismatches, and that it is the repairing, not perfection, that builds trust and resilience (Tronick and Gianino, 1986).

THE STEPS OF REPAIR

1. **Settle yourself.** Ground, and breathe.
2. **Name what happened,** simply: "That was loud." "That was sudden."
3. **Say whose it was:** "It was not about you." "It was not your fault."
4. **Reconnect:** your hand, your voice, your song.
5. **Answer the baby's question again,** with a yes: "You are welcome. You are safe. I am here."

Repair, month by month

MONTH 1 · THE BEGINNING **Grounding**
If her beginning was not what you hoped, ground yourself, rest a hand on your belly, and tell her: "You were a surprise, and now I am choosing you. You are wanted."

MONTH 2 · THE DISCOVERY **Repair words for the news**
Return to these as often as you need: "When we found out, we were scared. That was about us, not you. You are welcome here."

MONTH 3 · TAKING ROOT **A daily welcome**
After a frightening scan or a hard day, let the welcome carry a repair: "That was a hard day. I am here now. You are safe with me."

MONTH 4 · THE FIRST FLUTTERS **Hands on the belly**
If days pass when you could not notice her, come back with your hands: "I was busy and far away. I am here now, and I feel you."

MONTH 5 · THE FIRST SOUNDS **A song for her**
After raised voices or a tense moment, sing her song, then say: "That was loud. It was not about you. You are safe."

MONTH 6 · THE RHYTHM **Three daily oxytocin breaks**
After a stretch of tension, the breaks are the way back: "It has been a hard time. I am finding calm again, for both of us."

MONTH 7 · LIGHT AND VOICES **Pacing**
If fear filled the room, at a visit or at home, slow down with her afterward: "That was frightening to hear. I am with you, and we will be cared for."

MONTH 8 · THE TURNING **Tell her what you would like**
If a decision came fast or something was done without warning, tell her after: "That was sudden. I am sorry we did not tell you first. You are held."

MONTH 9 · RIPENING **Tell her about the birth to come**
If birth will be helped along or scheduled, tell her before and again after: "We chose to help you come today. You are welcome now, just as you are."

LABOR · THE THRESHOLD **Tell her what is happening**
If labor was long or interrupted, tell her after: "That was a long, hard journey, and you made it. I am so proud of you."

BIRTH · THROUGH THE GATE **Repair after intensity**
Use it whenever the birth was hard: "That was a lot. It is over. You are safe now. It was not your fault."

FIRST HOUR · THE WELCOME **Tell her the story of her birth**
If you were apart after birth, hold her close and tell her: "We were apart, and I missed you. I am here now, and I am so glad you are mine."

The First Trimester



Beginnings

Life is often present before anyone knows it. These first weeks are full of rapid building, and of big feelings as the news arrives.

Your baby. From a cluster of cells to a moving fetus. Her heart begins to beat, every major organ begins, and the placenta takes root.

You. Hormones rise quickly, bringing fatigue, nausea, and tenderness. The news can bring joy, fear, and old feelings to the surface.

Your circle. Let the people close to you ease your load, welcome her with you, and keep you company through the first visits.

Through the lens. She is present before anyone knows she is there. How she is welcomed, in thought, word, and feeling, is among the first things she takes in.

The focus. Welcome her, find one way to settle yourself, and gather the people who will support you.

When to read. As early as you can, even before you feel ready.

CARDS IN THIS SECTION

1 · The Beginning 44
2 · The Discovery 45
3 · Taking Root 46



Month 1 · The Beginning

Your baby

Pregnancy is counted from the first day of your last period, so conception happens around week 2. About a week later, the tiny ball of cells settles into the lining of your womb, and the cells that will become the placenta begin reaching toward your blood supply. In the pre- and perinatal view, her experience begins here, often before anyone knows she is there.

You

Hormones begin shifting at implantation. A missed period around week 4 is often the first sign, and some mothers notice fatigue, tender breasts, or light spotting. Life goes on as usual until the news arrives, and when it does, feelings can come all at once.

Your circle. Let the people close to you ease your load and meet your news with care. Your rest is her first world.

Imprints and what helps

The circumstances of conception, and whether she was hoped for, may become part of her earliest story. That story can be met without judgment. The most helpful foundation is simple: one way to settle yourself, such as grounding, and a circle of people you can turn to. If you have known loss or a long road to conceive, hope and fear can arrive together, and both are welcome.

BABY'S QUESTION Am I wanted?

Her life begins with how she was conceived and hoped for.

TO EXPLORE FURTHER

David Chamberlain describes the prenate as a conscious, sentient being, and gathered memories of the womb and birth shared by young children and adults. Ask yourself how you might welcome your baby as someone, not something, from the very start.



Month 2 · The Discovery

Your baby

Her heart begins to beat around week 6. The neural tube, the start of her brain and spinal cord, closes, and arm and leg buds appear. By week 8 every major organ system has begun, and the first touch receptors form around her mouth. She is already taking in how she is received.

You

Pregnancy hormones climb steeply, and nausea, deep fatigue, and a sharper sense of smell often peak now. Blood volume begins to rise. For many this is the month of discovery. Joy, shock, fear, and ambivalence can arrive together, and pregnancy may bring you closer to your own childhood, letting old feelings rise.

Your circle. Ask for help through the nausea and fatigue, and let the people around you welcome her with you.

Imprints and what helps

How the news was first received may stay with her. In one study, mothers' first reactions matched feelings their grown children later recalled. Conflict with partners or parents can reach her too. What helps is a welcoming response from the people around you, and, if the news was hard, simple repair words spoken to her. Waiting for first scans is full of uncertainty. Hard feelings are natural and need no guilt.

BABY'S QUESTION Am I welcome?

The news of her lands in your body, and she feels it.

TO EXPLORE FURTHER

Your baby senses your emotions through your body's chemistry, and later through your heartbeat, breath, and voice. Everyday stress is buffered by the placenta. It is long, unrelieved stress that matters most, and each return to calm helps (Ham and Klimo; Raffai).



Month 3 · Taking Root

Your baby

Now called a fetus, she moves, stretches, hiccups, and swallows amniotic fluid long before you can feel it. Sensitivity to touch spreads from her face across more of her body. Some of your chemistry, including stress hormones, reaches her through the placenta.

You

Nausea often eases near the end of this month as the placenta takes over hormone production, and your uterus begins to rise out of the pelvis. First prenatal visits and screenings usually happen now, and many families wait until now to share the news. Old feelings from childhood can surface in this window.

Your circle. Have someone with you at screenings and while you wait for results. A calmer home reaches her too.

Imprints and what helps

Your own unresolved history, and the chemistry of your states, can reach her. What helps is a safe, trauma-informed space where you can share your story if you choose, and support when it feels big. How you nourish and rest yourself shapes her lifelong health, and small, steady steps count. It is also a good time to begin looking for a doula, since many book months ahead. Worry while waiting for screening results is natural, not a failure of trust.

BABY'S QUESTION Is it safe to be here?

Your chemistry now reaches her through the placenta.

TO EXPLORE FURTHER

Fetal programming, the idea that conditions in the womb shape lifelong health (Nathanielsz). The three models of care, technocratic, humanistic, and holistic (Davis-Floyd), and the Mother-Friendly Childbirth Initiative, to help you choose the care that fits you.

The Second Trimester



The Middle

Your baby begins to hear, you begin to feel her, and the relationship becomes something you can both sense.

Your baby. Her movements grow coordinated, hearing begins around weeks 18 to 20, and she begins to respond to voices, touch, and sound.

You. Energy often returns and your belly shows. Feeling her move can turn an idea into a relationship, and your thoughts begin to turn toward birth.

Your circle. Invite others to notice her with you, let more loving voices reach her, and ask for the rest you need.

Through the lens. Her senses are opening, and relationship now runs both ways. When her movements and sounds are met, she learns she is heard.

The focus. Talk, sing, and answer her movements. Choose the stories you take in, and begin exploring your own birth story.

When to read. Around weeks 10 to 13, just before this trimester begins.

CARDS IN THIS SECTION

4 · The First Flutters 48
5 · The First Sounds 49
6 · The Rhythm 50

Month 4 · The First Flutters

Your baby

Her movements grow smoother and more coordinated, and she makes facial expressions. Her inner ear and taste buds are taking shape. Her movements meet the walls of the womb, and soon they will meet your hand.

You

Energy often returns, and your belly begins to show. The placenta now makes most of the hormones of pregnancy. Some mothers, especially those who have been pregnant before, feel the first flutters. Relief and steadiness are common, and your sense of yourself begins to shift toward motherhood.

Your circle. Invite someone to notice her movements with you, and to shield you from frightening birth stories.

Imprints and what helps

Whether her movements are noticed and met may be one of her earliest lessons in relationship. Simple connection practices, for you and your partner, help, and a bonding program such as Pregnancy Dialogues can deepen them. As your pregnancy shows, others may share birth stories, many of them fearful. You can choose what you take in, set kind boundaries, and gather stories of births that went well.

BABY'S QUESTION When I reach out, will someone meet me?

Her movements are her first way of reaching out.

TO EXPLORE FURTHER

Prenatal attachment, which grows as you turn your attention toward your baby. Karen Strange teaches that a baby's movements are her way of telling her story, and asks: can you slow down and be present to hear what she is saying?



Month 5 · The First Sounds

Your baby

Hearing begins. The first sounds reach her around weeks 18 to 20, and her hearing keeps sharpening through about week 25. Fine hair and a protective coating of vernix cover her skin, and sleep and wake cycles appear. Voices, music, laughter, and raised voices reach her as vibration through the womb wall.

You

Most mothers now feel movement. Relaxin softens the ligaments, and round ligament aches are common. The anatomy scan usually happens around week 20. Feeling her move can turn an idea into a relationship, and your partner can join in: a hand on the belly when she moves, a word or a song in answer.

Your circle. Let more loving voices reach her, and bring someone with you to the anatomy scan.

Imprints and what helps

The sound world of your home may stay with her. In one study, grown children recalled music, laughter, and yelling their mothers also remembered. What helps is time to talk, sing, and play with her, and repair after loud or tense moments. If the anatomy scan brings unexpected news, you can ask what a result means, learn the options, and take time before deciding. Fear or grief is a natural response.

BABY'S QUESTION Is my world a kind place?

Hearing begins, and the sounds of home reach her.

TO EXPLORE FURTHER

Your voice, carried through your own body, is the loudest and clearest sound she hears. Ham and Klimo describe a father whose "hoo, hoo" game from the fifth month led his newborn to stop nursing and turn toward his voice.



Month 6 · The Rhythm

Your baby

She begins to respond to sound, often moving or startling at a sudden loud noise. Her lungs begin making surfactant and her brain grows quickly. Her sleep is not yet divided into stages: she moves through loose cycles of rest and activity, and her brain waves are six to ten times slower than an adult's.

You

Your belly grows quickly. Back aches, heartburn, and swelling may begin, and a glucose screening is usually offered between weeks 24 and 28. Your thoughts may begin turning toward the birth, and your own birth story, and your mother's, may come to mind.

Your circle. Ask for real rest, and let others take on tasks. The calm rhythm of your home is the world she feels.

Imprints and what helps

The emotional climate over time matters most: ongoing tension or neglect, or warmth and contact. What helps is a steady rhythm of calm, a childbirth class that fits you, the doula you have found or are still seeking, and the rest you need, asked for in plain words. If the screening leads to a diagnosis such as gestational diabetes, it is information to guide your care, not a judgment on your body. Disappointment is natural, and it can sit beside caring well for yourself.

BABY'S QUESTION Can I rest here?

Her first rhythms of rest take shape in the climate around her.

TO EXPLORE FURTHER

William Emerson's "elephant in the birthing room": everyone brings a birth story into the birth room. Oxytocin, the hormone of calm and connection, is easier to find in labor when it has been practiced in pregnancy (Strange).

The Third Trimester



The Ripening

Your baby grows, space grows tight, and both of your bodies prepare for birth.

Your baby. Her eyes open, dreaming sleep appears, and she comes to know your voice. Many babies settle head down, and her own lungs may help signal when labor begins.

You. Sleep grows harder and practice contractions stronger. Anticipation grows, and so can fear. Choices about position, monitoring, and induction may arise.

Your circle. Gather people who hear your fears without adding to them, plan help for after the birth, and let someone share the waiting.

Through the lens. She is preparing for birth alongside you. How choices are made, and the feeling in the room, may become part of how she arrives.

The focus. Slow down, plan your birth space and support, practice staying in the decisions, and prepare for flexibility, not fear.

When to read. Around weeks 24 to 27, just before this trimester begins.

CARDS IN THIS SECTION

7 · Light and Voices 52
8 · The Turning 53
9 · Ripening 54



Month 7 · Light and Voices

Your baby

Her eyes open and can sense light through your belly. She is gaining fat, practicing breathing, and moving between clear sleep states. REM sleep appears now, the active sleep linked with dreaming, and she spends much of each day in it. The voices she hears most will be familiar after birth.

You

Your blood volume is rising toward nearly half again its usual amount, and she crowds your lungs and stomach. Sleep grows harder, breath shorter, and practice contractions more noticeable, and visits move to every two weeks. Anticipation grows, and so can fear of birth.

Your circle. Choose people who hear your fears without adding to them, and help your partner ready their voice for her.

Imprints and what helps

Fear in the room can become part of the birth: your own, your family's, or a provider's past cases. What helps is planning your birth space and choosing who will speak up for you; a doula can be a strong advocate at a hospital birth. It also helps to begin preparing for flexibility, not fear. Exploring other paths, such as a longer labor, an induction, or a cesarean, lets you meet a change as a choice rather than a collapse of the plan.

BABY'S QUESTION **Can I trust those around me?**

She knows the voices near her, and feels the fear or calm in them.

TO EXPLORE FURTHER

Sarah Buckley's four hormone systems of birth: oxytocin for contractions and love, endorphins for easing pain, adrenaline for the final pushes, and prolactin for mothering. Your rights in labor: companions, freedom to move, food and drink, and informed consent (MFCI).



Month 8 · The Turning

Your baby

She gains weight quickly and space grows tight, so her movements change from tumbling to stretching and pushing. Studies first show her responding to your voice differently from other voices, and after birth she prefers it. Antibodies cross the placenta to prepare her immune system, and many babies settle head down now.

You

Your uterus presses up under your ribs and builds oxytocin receptors for labor, and your breasts may begin leaking colostrum. Nesting energy often arrives. If she is breech, a real decision lies ahead, and you deserve full information and room to choose.

Your circle. Plan help for after the birth, and keep your bond with your partner warm. She is held inside it.

Imprints and what helps

As space grows tight, she may feel held or hemmed in, and how decisions about her position are made may shape her experience, and yours. What helps is gentle movement, telling her what is happening, and practicing how you will stay in the decisions: asking about benefits, risks, and alternatives, taking a moment, and saying yes or no. If she comes early, skin-to-skin care in the NICU keeps her close. A baby doula and help planned for the weeks after birth are worth arranging now.

BABY'S QUESTION **Will I be held as space grows tight?**

Room grows tight, and she begins to feel the walls holding her.

TO EXPLORE FURTHER

Breech options: waiting, gentle positioning, an external version, or a vaginal breech birth with a skilled provider (Fischbein and Freeze). Continuous monitoring in low-risk labors is linked to about 20 percent more cesareans; you can ask about listening in at intervals.



Month 9 · Ripening

Your baby

Her lungs finish maturing, and full term arrives at 39 to 40 weeks. Research suggests her own maturing lungs release a signal that helps start labor, so she may have a part in deciding when birth begins.

You

She may drop lower, easing your breath but adding pressure. Practice contractions grow stronger and your cervix begins to soften. Waiting can be hard, especially as pressure to induce grows, and you may feel ready, impatient, or afraid, sometimes all in one day.

Your circle. Let someone share the waiting and answer the questions, so you can turn inward and trust her timing.

Imprints and what helps

Being waited for, or hurried, may become part of her story. Labor usually begins when you are both ready, and her own body may help signal when. An induction is a choice to weigh, not an automatic step, and if one is chosen, telling her what is coming can soften it. What helps is an informed, unhurried decision, continuous support planned ahead, and a conversation about what you would want if plans change.

BABY'S QUESTION Will I be given time?

Her own body helps signal when she is ready to be born.

TO EXPLORE FURTHER

The ARRIVE trial and elective induction at 39 weeks. Synthetic oxytocin does not act like your own and is linked to more postpartum depression and anxiety (Kroll-Desrosiers). Expressing colostrum after 37 weeks, with your provider's approval, helped milk flow sooner after birth (Singh).

Birth

The Threshold



Hormones, the space, and the people present all shape how your baby arrives and how she is met.

Your baby. Labor holds her in rhythm and pressure. Birth brings light, air, and sound all at once. The first hour brings your skin, your smell, and your voice.

You. Oxytocin, endorphins, adrenaline, and prolactin carry you through, and they flow best when you feel safe, private, and unobserved.

Your circle. Someone who stays the whole way, a familiar voice for you and for her, and a quiet, protected golden hour.

Through the lens. Birth is her first great passage and her first welcome. Even when plans change, telling her what is happening and holding her close can soften what she meets.

The focus. Guard the space, tell her what is happening, and keep her close. If plans change, stay in the decisions and offer repair.

When to read. Early in the third trimester, around weeks 28 to 34, so you have time to practice and plan before the birth window opens at 37 weeks.

CARDS IN THIS SECTION

10 · The Threshold 56
11 · Through the Gate 57
12 · The Welcome 58

Labor · The Threshold

Your baby

Contractions press and release her whole body like a long, firm embrace. Her own stress hormones surge, protecting her through each squeeze and preparing her lungs, and she receives some of your oxytocin and endorphins. Pressure, rhythm, and pause: the pace of labor becomes part of her story.

You

Oxytocin brings rhythmic contractions, and beta-endorphin eases pain and carries you inward. You labor best when you can move, eat and drink lightly, and follow your body, and when you feel private, safe, and unobserved, much like lovemaking.

Your circle. Have someone stay the whole way and guard a quiet space, telling her, too, what is happening.

Imprints and what helps

The pace and pressure of labor may stay with her: moments of feeling stuck, and a journey supported, or interrupted by synthetic oxytocin, forceps, or vacuum. What helps is a guarded space: dim light, quiet, trusted people, and no routine procedures without a reason. Pain is part of labor; suffering is not, and choosing relief needs no guilt. If plans change, three things help you stay present: someone by your side, clear explanations, and time to respond.

BABY'S QUESTION Can I make it through?
Labor presses her whole body, wave after wave.

TO EXPLORE FURTHER

Sarah Buckley's "undisturbed birth": when labor feels watched or unsafe, adrenaline rises and can slow oxytocin, and with it labor. Ina May Gaskin reminds us that labor is part of nature, and that walking and eating belong in it.



Birth · Through the Gate

Your baby

In the final push, a surge of noradrenaline makes her alert, often with wide open eyes at your first meeting. She takes her first breath, and her circulation changes course within minutes. Light, air, sound, and gravity arrive all at once.

You

Near the end, a surge of adrenaline gives you strength for the final pushes, the fetal ejection reflex. Many mothers feel a burst of energy and alertness to meet the baby. Pain medication, such as an epidural or opioids, can soften this surge, so you may both be less alert when you first meet.

Your circle. Keep a familiar voice with you and with her, so neither of you is alone if plans change.

Imprints and what helps

Every physiological birth event may leave an imprint, and harsh lights, noise, routine suctioning, and tight wrapping can overwhelm a newborn. What helps is a calm arrival, delayed cord clamping, and a gentle cesarean if surgery is needed. If the birth takes an unexpected turn, she can be told what is happening, held close as soon as possible, and offered repair words afterward. A birth that changed course is not a failure.

BABY'S QUESTION Is it safe to arrive?

Light, air, and sound meet her all at once.

TO EXPLORE FURTHER

A vaginal birth helps clear her lungs and seeds her with your beneficial microbes; skin to skin and breastfeeding help close the gap after a cesarean. Thomas Verny on birth and early imprints. Karen Strange on rupture and repair.

First Hour · The Welcome

Your baby

Placed skin to skin, she warms, steadies her heart and breathing, and may crawl toward the breast on her own. Skin, smell, heartbeat, and your voice, now from the outside. A newborn may not know she is out, or that she made it, until she is told and held.

You

Skin to skin, oxytocin and prolactin peak. Oxytocin helps the placenta release and protects against bleeding, and prolactin begins milk and mothering. Awe, relief, exhaustion, and the first look at your baby may arrive together.

Your circle. Protect a quiet golden hour, so she is welcomed skin to skin, and by more than one person.

Imprints and what helps

Welcome or separation may be among her first lessons about the world. What helps is keeping you together: skin to skin, breastfeeding, and rooming-in, with newborn checks on your chest and the first bath waiting. If the golden hour does not happen, through separation or medical care, the bond is not lost. Relationships are built over time, and repair can begin as soon as you are together. Mixed feelings in the days after are natural and need no guilt.

BABY'S QUESTION Do I belong here, with you?

Skin, smell, and voice tell her where she has arrived.

TO EXPLORE FURTHER

Humanistic care keeps mother and baby together; taking the baby to a nursery for bathing is not part of this model (Davis-Floyd). Watch in the weeks after for lingering distress, such as flashbacks, numbness, or avoidance, and reach out for support (Seng and Taylor).

When the path is harder

No pregnancy follows the map exactly. Some begin in ways this guide does not picture, and some carry grief, fear, or old wounds alongside the hope. Whatever your path, the center of this guide still holds: she is here, she is listening, and connection and repair are possible.

CONCEIVING WITH HELP

IVF, donor eggs or sperm, or a surrogate can make a beginning feel clinical, or full of waiting and loss. Many in this field invite parents to tell the baby the story of how she came to be, and how long and how much she was wanted. If a donor or surrogate is part of her story, it can be told with gratitude, and at her pace as she grows.

WITHOUT A PARTNER, OR WITHOUT SUPPORT

You may be single by choice, separated, or with a partner who is not involved. Her first world is you and the people you gather around you: friends, family, a doula, a community. The circles of support on page 67 are a place to start.

WHEN HOME IS NOT SAFE

Abuse can begin or grow worse in pregnancy, and it reaches your baby as well as you. You deserve safety. In the United States, the National Domestic Violence Hotline is open day and night: call 1-800-799-7233, or text START to 88788. A doula, a care provider, or a friend can help you make a plan.

Loss, and hope after loss

PREGNANCY AFTER LOSS

If you have lost a pregnancy or a child before, hope and fear often arrive together, and letting yourself attach can feel risky. This is natural. Some parents tell the new baby about her brother or sister, so the loss is held in the open rather than in silence. She is not a replacement. She is herself.

WHEN THIS PREGNANCY BRINGS HARD NEWS

A difficult diagnosis, a twin who does not continue, or a pregnancy that ends: these are griefs that deserve time and support. Your baby can still be spoken to, held in your attention, and named. Perinatal loss and perinatal hospice support exist for exactly these paths.

CARRYING A BABY YOU WILL NOT RAISE

If you are a surrogate, or carrying a baby toward adoption, she is still listening to you now. Many find it meaningful to speak to her honestly, about who will hold her and that she was carried with care. Your own feelings, including grief, deserve support too.

ADOPTING A BABY

If you are adopting, your baby will arrive with a story that began before you, including the loss of the first voice she knew. Telling her that story, and meeting her early grief with patience, is a gift (Verrier, *The Primal Wound*).

GRIEF HAS ITS OWN TIMING

Grief may come in waves: at scans, due dates, and anniversaries. Partners often grieve differently, and both ways are real. Groups such as Share Pregnancy and Infant Loss Support, and Postpartum Support International, offer companionship along the way.

When birth takes a hard turn

A BIRTH THAT WAS HARD

An emergency cesarean, a frightening labor, separation, or feeling unheard can leave a mark on parents as well as babies, and many people carry a hard birth quietly for years. Telling the story to someone who listens, and telling your child what happened, are both forms of repair. Approaches such as EMDR and birth story listening can help (page 65).

EARLY ARRIVALS AND THE NICU

If she comes early or needs special care, you are still her first world. Your voice, your smell, skin to skin as soon as it is allowed, and your hand through the incubator all reach her. Ask her nurses what you can do.

WHEN THE PREGNANCY WAS NOT PLANNED

An unexpected pregnancy can bring shock, ambivalence, or fear. Mixed feelings are not a verdict on your love. Telling her, "This was a surprise, and I am finding my way to you," is honest, and a beginning.

Looking back with kindness

Learning about imprints can bring up the past: a pregnancy you wish had gone differently, an older child's birth, or a pregnancy you ended. This knowledge is not a scorecard.

AFTER AN ABORTION

If you have ended a pregnancy, reading this may stir grief, relief, regret, or all of these at once. You made a decision with what you knew and what you had at the time. Grief and love can sit beside that decision. Some people find comfort in a private ritual of acknowledgment, or in talking with someone who listens without judgment, such as All-Options, which supports people through every pregnancy experience.

IF YOU CARRY EARLIER TRAUMA

Past abuse or assault can make exams, birth, and being touched hard. You can tell your care providers what helps you feel safe, and ask for trauma-informed care. A therapist can walk with you before the birth.

REPAIR IS POSSIBLE AT ANY AGE

If you look back at an older child's beginning and see what was missing, it is not too late. Children of every age, and adults too, can be told their story with kindness, and repair can begin in any relationship, including the one with yourself.

GOING DEEPER · REGRESSION

Memories that return

For a century, people in therapy have described experiences that seem to come from before words: a sense of the womb, of being born, of how they were received. In pre- and perinatal psychology these are called regression experiences, early impressions returning to conscious awareness.

WHAT HAS BEEN GATHERED

> From the 1960s, psychiatrists Frank Lake and Stanislav Grof recorded clients reliving womb and birth experiences in deep therapeutic states.

> Obstetrician David Cheek, working with hypnosis, described adults recalling the movements and circumstances of their own births.

> David Chamberlain compared the birth memories of mothers and their children, recalled separately under hypnosis, and found many details matched (*Babies Remember Birth*).

> Ham and Klimo found that adults' recalled sense of their mothers' feelings in pregnancy matched what their mothers reported.

> Surveys in Japan found many young children speaking, unprompted, of the womb or their birth (Ikegawa).

A careful note: memory recalled in hypnosis or deep states can be shaped by suggestion and expectation, and mainstream science doubts that memories like these can form so early. What these accounts are is still being explored.

How early memory may be held

How could experience from before birth be carried? Part of the answer may be implicit memory: patterns of feeling held in the body and nervous system before there are words for them. Some practitioners speak of cellular or energetic memory, something carried that research has not yet found a way to measure.

However it is held, what people describe is often felt in the body first: in breath, in posture, in emotions that seem older than the story they know.

PREGNANCY AS A DOORWAY

Carrying a child often stirs a parent's own earliest story. Feelings may rise that seem bigger than the moment, or familiar in a way that is hard to name: a fear of being left, a longing to be held. In this lens, such moments may be your own early imprints surfacing, a natural part of becoming a parent.

Meeting them gently matters for you both. What is felt and understood can be soothed, and what is soothed is less likely to be passed on.

The healing practice on the back of each card is a place to begin. If what surfaces feels larger than you can hold, the people on page 65 work with exactly these early experiences.

GOING DEEPER WITH SUPPORT

People who can walk with you

The healing practices on the cards are gentle starting points. When something stirs more than you can hold, or you want to go further, these people and approaches can help.

PRACTITIONERS

Pre- and perinatal practitioners and educators work with bonding, early imprints, and birth stories. APPPAH lists trained practitioners.

Perinatal mental health specialists support anxiety, depression, and trauma in pregnancy and after. Look for the PMH-C credential, through Postpartum Support International.

Trauma-informed psychotherapists help with history that surfaces, at a pace that feels safe.

Doulas for birth and the weeks after, and a baby doula to follow your baby's journey.

Childbirth educators, bonding programs such as Pregnancy Dialogues, and **lactation consultants** (IBCLC).

MODALITIES

Prenatal and birth therapy, in the tradition of Ray Castellino and William Emerson, works with the imprints of conception, womb life, and birth, for babies, children, and adults.

Somatic Experiencing gently releases stress and trauma held in the body.

Craniosacral therapy offers light, hands-on support for parents and newborns.

EMDR helps the mind process hard memories, including a difficult birth.

Birth story listening helps you tell your story, be heard, and find meaning in it.

An educator can help you find the right fit. Choose people who listen, who leave you feeling safe, and who let you set the pace. Keep their details on the next two pages.

Help, reading, and sources

IF YOU NEED SUPPORT NOW (UNITED STATES)

National Maternal Mental Health Hotline: call or text 1-833-TLC-MAMA (1-833-852-6262).

Postpartum Support International HelpLine: call or text 1-800-944-4773.

National Domestic Violence Hotline: call 1-800-799-7233, or text START to 88788.

In a crisis, call or text 988. In an emergency, call 911.

FURTHER READING

Chamberlain, D. *Windows to the Womb*; *Babies Remember Birth*.

Verny, T., and Kelly, J. *The Secret Life of the Unborn Child*.

Buckley, S. *Gentle Birth, Gentle Mothering*.

Gaskin, I. M. *Ina May's Guide to Childbirth*.

Siegel, D., and Hartzell, M. *Parenting from the Inside Out*.

Strange, K. *Simple Tools for Mothers*.

GOING DEEPER WITH SUPPORT

My circles of support

Three rings of people around you and your baby.



Ring one · Care providers

Midwife, doctor, doula, lactation consultant, pediatrician

1 NAME _____ PROFESSION _____

CONTACT _____

2 NAME _____ PROFESSION _____

CONTACT _____

3 NAME _____ PROFESSION _____

CONTACT _____



Ring two · Family and friends

The people who will cook, listen, hold the baby, and hold you

4 NAME _____ RELATIONSHIP _____

CONTACT _____

5 NAME _____ RELATIONSHIP _____

CONTACT _____

6 NAME _____ RELATIONSHIP _____

CONTACT _____



Ring three · Pre- and perinatal support

Pre- and perinatal practitioners and educators, prenatal and birth therapy, Somatic Experiencing, craniosacral therapy, birth story listening, bonding programs

7 NAME _____ MODALITY _____

CONTACT _____

8 NAME _____ MODALITY _____

CONTACT _____

9 NAME _____ MODALITY _____

CONTACT _____

10 NAME _____ MODALITY _____

CONTACT _____

11 NAME _____ MODALITY _____

CONTACT _____

12 NAME _____ MODALITY _____

CONTACT _____

NOTES _____

Words in this guide

Pre- and perinatal	Before birth, and around birth: from conception through the first year.
Prenate	A baby before birth.
Imprint	A lasting impression left by early experience, held in the body and nervous system before there are words.
Baby's question	A quiet question that may begin to emerge for the baby, such as "Am I welcome?" It stays open, and each repair answers it again.
Explicit memory	Memory we can recall and put into words: facts, events, stories.
Implicit memory	Memory held without a sense of remembering: in the body, in feelings, in what we expect.
Regression	Early impressions returning to awareness, often in deep or therapeutic states.
Placenta	The organ that nourishes her and carries some of your chemistry to her, while buffering much of it.
Epigenetics	How surroundings change the way genes are switched on and off, without changing the genes themselves.
Cortisol	A stress hormone. Brief rises are normal; long, unrelieved stress matters most.
Oxytocin	The hormone of calm, closeness, and labor contractions.

GOING DEEPER

Nervous system states	In polyvagal theory: safe and connected, mobilized, and shut down.
Neuroception	The body's wordless, constant scanning for safety and danger.
Window of tolerance	The range in which you can feel strongly and still think and connect. Also called a window of capacity.
Self-regulation	Finding your own way back to steadiness.
Co-regulation	Steadying through another person. A baby borrows steadiness from her parents.
Resource	Anything that helps your nervous system feel steadier: a person, a place, a memory, a sensation.
Rupture and repair	A break in connection, and the return that heals it. Repair builds trust.
Healthy separateness	Telling her which feelings are yours, so she need not carry them as her own.
Trauma	An experience that was too much, too fast, or too soon, with too little support.
Resilience	The capacity to meet difficulty and find the way back to balance.
Circle of support	The people who hold you so you can hold your baby. Each of you needs at least two layers (Castellino).
Doula	A trained companion who supports the mother through birth and the weeks after.
Baby doula	Someone who follows the baby's journey through birth.
Informed consent	Your right to understand the benefits, risks, and alternatives of any care, and to say yes or no.
The golden hour	The first hour after birth, when skin to skin and closeness matter most.

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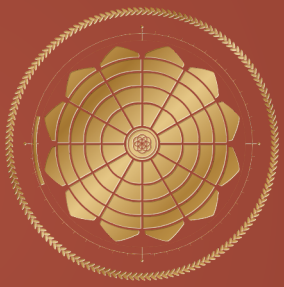
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NOTES

What I am noticing

Her movements, your feelings, what surfaced, what you want to remember.



At the center of all of this is a child who is already present, already listening, and already being shaped by the world around her.

A card deck and guidebook for the journey of pregnancy and birth, drawn from the Pregnancy and Birth Mandala and the lens of pre- and perinatal education.

Created by Erin Singleton as part of APPPAH's Prenatal & Perinatal Educator certification.

An educational companion, not medical advice. Talk with your care provider about your own pregnancy.